6.8 Ulcerative Colitis*

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • "I pass bloody stools 8 times a day." • "I rush to the toilet." **Objective** • Bloody diarrhea • Lower abdominal cramps • Hyperactive bowel sounds	**ACTUAL** Impaired gastrointestinal motility related to inflammation of the colonic mucosa as evidenced by frequent bloody stools and urgency.	• Fewer stools and less bleeding within 1 week of treatment.	• Record frequency, consistency and any blood. • Give corticosteroids and aminosalicylates as prescribed. • Keep a commode or toilet close. • Protect the perianal skin. • Advise avoiding foods that make it worse.	• Goal reached as stool frequency decreases.
Subjective • "I feel dizzy when I stand." **Objective** • HR 112/min • Hb 9 g/dL • Dry mouth	**ACTUAL** Inadequate fluid volume related to fluid and blood loss in stool as evidenced by tachycardia, dry mucosa and low hemoglobin.	• Vital signs stable and urine output adequate within 24 hours.	• Give IV fluids and blood as prescribed. • Keep an intake and output chart. • Follow potassium and hemoglobin.	• Met when vital signs are stable.
Subjective *No specific complaint* **Objective** • Severe colitis • High-dose corticosteroids	**RISK** Risk for physical injury as evidenced by risk factors: toxic megacolon and perforation in severe colitis.	• Complications picked up early.	• Measure abdominal girth daily. • Report sudden distension, fever, tachycardia, or fewer stools with pain. • Avoid antidiarrheals and opioids unless ordered.	• Achieved if no complication develops.

* **References for this care plan:** [32], [35], [37], [91]. Full entries are listed in the References section of the CPMN book.